

Patient:

Referring Physician:

UUID: 1A79B4F7-8315-450C-A1E9-BAA89C6E9001
TCGA-FY-A3R8-01A-PR

Redacted



DOB: Age: Gender:

Ref#: Hosp#: Provider Group:

Date of Service: Date Received: Outpatient Case #:
Room: Bed: Date Reported:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

ICD-O-3
carcinoma, papillary, thyroid
8260/3

Site: thyroid
C73.9

A. THYROID GLAND, LEFT LOBE, LOBECTOMY:

- PAPILLARY THYROID CARCINOMA, CLASSICAL VARIANT.
 - 1.7 cm in maximum dimension.
 - Tumor demonstrates papillary, follicular, and cribriform architecture, with classical cytomorphology.
- PERINEURAL INVASION: NOT IDENTIFIED.
- EXTRATHYROIDAL EXTENSION: PRESENT.
 - Tumor invades perithyroidal adipose tissue and skeletal muscle.
- MARGINS: NEGATIVE.
 - Tumor is 0.03 cm from the posterior margin.
- MULTIPLE ADENOMATOID NODULES (MULTINODULAR GOITER).
- SEE TUMOR STAGING SUMMARY BELOW.

3-30-12 RD

B. THYROID GLAND, RIGHT LOBE AND ISTHMUS, LOBECTOMY:

- MULTIPLE ADENOMATOID NODULES (MULTINODULAR GOITER).
- ONE HYPERCELLULAR PARATHYROID GLAND (SEE COMMENT).
- NEGATIVE FOR MALIGNANCY.

C. CENTRAL NODE TISSUE, EXCISION:

- BENIGN THYMIC TISSUE.
- NEGATIVE FOR METASTATIC CARCINOMA.

PATHOLOGIC TUMOR STAGING SUMMARY:

Tumor type: Papillary thyroid carcinoma.
Primary tumor: pT1b (1.7 cm in greatest dimension, with minimal extrathyroid extension).
Regional lymph nodes: pNX (none present for evaluation).
Distant metastasis: Not applicable.
Pathologic stage: I.
Lymphovascular invasion: Not identified.
Margin status: R0 (0.03 cm from posterior left lobe margin).

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
IPAA Discrepancy		X
Prior Malignancy History		X
Local/Synchronous Primary Noted		X
Age (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	3/3/12	3/2/12
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COMMENT: The parathyroid tissue identified in part B of this case may represent normal parathyroid tissue that is relatively hypercellular, or parathyroid adenoma. Clinical correlation is suggested.

The patient's previous fine needle aspiration cytology diagnosis of papillary thyroid carcinoma of the left lobe () is noted, and correlates with the current specimen.

Tumor Staging Information

Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, and CAP protocol.

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Procedure:

Specimen type:

Specimen integrity:

Specimen size:

TUMOR FEATURES:

Tumor size:

Tumor focality:

Tumor laterality:

Histologic type:

Lymphovascular invasion:

Tumor capsule:

Tumor capsular invasion:

Extrathyroidal extension:

LYMPH NODES:

Margin evaluation:

Distance to closest margin:

Other margins:

PATHOLOGIC TUMOR STAGING DESCRIPTORS:

Primary tumor:

Regional lymph nodes:

Distant metastasis:

Margin status:

Pathologic stage:

Additional pathologic findings:

Comment:

Total thyroidectomy with central compartment dissection.

Thyroid gland, with central compartment tissue.

Multiple portions of tissue, with each lobe and the central compartment submitted separately.

Left lobe: 3.8 x 2.2 x 1.3 cm.

Right lobe: 4.2 x 2.2 x 1.7 cm.

Isthmus: 1.4 x 1.3 x 0.8 cm.

Greatest dimension: 1.7 cm.

Additional dimensions: 1.4 x 0.9 cm.

Unifocal.

Left.

Papillary thyroid carcinoma, classical variant.

Not identified.

Partially encapsulated.

Present.

Present (tumor extends into perithyroidal adipose tissue and skeletal muscle).

None present for evaluation.

Negative (R0).

0.03 cm from the posterior left lobe margin.

Uninvolved by invasive carcinoma.

pT1b (1.7 cm in greatest dimension, extending into perithyroidal adipose tissue and skeletal muscle).

pNX.

Not applicable.

R0 (0.03 cm from the posterior left lobe margin).

I.

Multiple adenomatoid nodules.

Hypercellular parathyroid tissue.

Not applicable.

MD, Pathologist

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FINAL SURGICAL PATHOLOGY REPORT

Signed

Source of Specimen:

- A. Thyroid; Left lobe of thyroid
- B. Thyroid; right lobe of thyroid and isthmus
- C. Thyroid; central node tissue of thyroid

Clinical History/Operative Dx:

Thyroid cancer

Gross Description:

A. Single specimen left lobe of thyroid, stitch marks cancer area. Initially received in the fresh state for Oncogenotyping studies is a 5 gram lobe of thyroid, 3.8 x 2.2 x 1.3 cm. The posterior and anterior lobular and slightly shaggy, red-tan surfaces are differentially inked black and blue, respectively. A palpable mass is noted at the likely superior pole marked with a suture. The suspicious areas are initially incised revealing a well-circumscribed, dense, light gray tumor mass measuring 1.7 x 1.4 x 0.9 cm and focally abutting the anterior and posterior thyroid margins. Using a sterile technique, representative portions of the tumor are retrieved centrally and submitted for Oncogenotyping studies. The remaining cut sections of the thyroid demonstrates meaty deep red parenchyma with an additional light gray nodule, 3 mm, abutting the anterior margin within 1.0 cm of the inferior pole, and an additional adjacent dark and tan follicular appearing nodule, 0.4 cm (A5). The lobe is entirely submitted for microscopic evaluation, in a sequential fashion, superior to inferior in A1-A6.

B. Part B is labeled right lobe of thyroid and isthmus. Received in formalin is an 11 gram lobe of thyroid, 4.2 x 2.2 x 1.7 cm with a 1.4 x 1.3 x 0.8 cm extension of the isthmus. The surface of the prostate (???) is predominantly glistening smooth and lobular, pink-red and is now differentially inked black and blue, posterior-anterior, respectively. The specimen is serially sectioned perpendicularly through the superior and inferior long axis revealing predominantly semi-translucent, gelatinous nodularity, up to 1.3 cm throughout with a suspicious light gray nodule, 0.5 x 0.4 x 0.3 cm, mid-lobe, within 0.2 cm of the anterior and 0.3 cm of the posterior thyroid margins (B3). An irregular, well-circumscribed, glistening, tan, follicular appearing nodule, 0.9 x 0.8 x 0.7 cm, abutting the inferior pole (B5). Representative sections of the thyroid are submitted in a sequential fashion, superior to inferior in B1-B5.

C. Part C is labeled central node tissue of thyroid. Received in formalin is a yellow-tan portion of fatty soft tissue, 2.8 x 1.7 x 0.8 cm. Examination reveals two lymph node candidates, 0.4 and 0.6 cm each. The lymph node tissue and residual fat is entirely submitted for microscopic evaluation.

Cassette summary: C1) two lymph node candidates, each bisected (one marked yellow), C2) remaining fat.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

C. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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END OF REPORT (FINAL)